

OMEZOLE 20 CAPSULE

VIOME06-var2 (MY)

DESCRIPTION

Light caramel opaque and flesh opaque capsule with "OMEZOLE 20" printed on one end and "hovid" printed on the other end of the capsule.

COMPOSITION

Each capsule contains: Omeprazole 20 mg.

PHARMACODYNAMICS

Omeprazole is activated at an acidic pH to a sulphenamide derivative that binds irreversibly to H⁺/K⁺ ATPase, an enzyme system found at the secretory surface of parietal cells. It thereby inhibits the final transport of hydrogen ions (via exchange with potassium ions) into the gastric lumen. Since the H⁺/K⁺ ATPase enzyme system is regarded as the acid (proton) pump of the gastric mucosa, omeprazole is known as a gastric acid pump inhibitor. Omeprazole inhibits both basal and stimulated acid secretion irrespective of the stimulus.

During treatment with antisecretory medicinal products, serum gastrin increase in response to the decreased gastric acidity. The increased CgA level may interfere with investigations for neuroendocrine tumours.

Available published evidence suggests that proton pump inhibitors should be discontinued between 5 days and 2 weeks prior to CgA measurements. This is to allow CgA levels that might be spuriously elevated following PPI treatment to return to reference range.

PHARMACOKINETICS

Omeprazole is rapidly absorbed, but not to a variable extent, following oral administration. Absorption of omeprazole is not affected by food. The absorption of omeprazole, as well as being formulation-dependent, also appears to be dose-dependent, as increasing dosage above 40 mg has been reported to increase the plasma concentrations in a non-linear fashion. Following absorption, omeprazole is almost completely metabolized in the liver, primarily by cytochrome P450 isoform CYP2C19. It is eliminated 72 to 80% through renal and 18 to 23% through fecal. In dialysis, it is not readily dialyzable because of extensive protein binding.

INDICATIONS

Omeprazole is indicated for:

- treatment of reflux oesophagitis.
- duodenal ulcer; benign gastric ulcer.
- long term treatment of pathologic gastric hypersecretion associated with Zollinger-Ellison syndrome.

CONTRAINDICATIONS

Contraindicated in patients known to be hypersensitive to omeprazole.

WARNINGS AND PRECAUTIONS

Before giving omeprazole to patients with gastric ulcers the possibility of malignancy should be considered since omeprazole may mask symptoms and delay diagnosis.

Omeprazole is extensively metabolized in the liver and some sources recommended that dosage should be reduced in hepatic impairment.

Risk-benefit should be considered when the following medical problems exist: chronic, current or history hepatic disease where dosage reduction may be required due to increased half-life.

Interference with laboratory test:

Increased Chromogranin A (CgA) level may interfere with investigations for neuroendocrine tumours. If the patient(s) are due to have a test on Chromogranin A level, Omeazole 20 Capsule treatment should be stopped for at least 5 days before CgA measurements to avoid this interference (see section Pharmacodynamics). If CgA and gastrin levels have not returned to reference range after initial measurement, measurements should be repeated 14 days after cessation of proton pump inhibitor treatment.

Regular Surveillance:

Patients on proton pump inhibitor treatment (particularly those treated for long term) should be kept under regular surveillance.

Subacute Cutaneous Lupus Erythematosus (SCLE):

Proton pump inhibitors are associated with very infrequent cases of subacute cutaneous lupus erythematosus (SCLE). If lesions occur, especially in sun-exposed areas of the skin, and if accompanied by arthralgia, the patient should seek medical help promptly and the health care professional should consider stopping Omeazole 20 Capsule. SCLE after previous treatment with a proton pump inhibitor may increase the risk of SCLE with other proton pump inhibitors.

Hypomagnesaemia:

Severe hypomagnesaemia has been reported in patients treated with PPI like Omeazole 20 Capsule for at least three months, and in most cases for a year. Serious manifestations of hypomagnesaemia such as fatigue, tetany, delirium, convulsions, dizziness and ventricular arrhythmia can occur but they may begin insidiously and be overlooked. In most affected patients, hypomagnesaemia improved after magnesium replacement and discontinuation of the PPI.

For patients exposed to be on prolonged treatment or who take PPI with digoxin or drugs that may cause hypomagnesaemia (e.g. diuretics), health care professionals should consider measuring magnesium levels before starting PPI treatment and periodically during treatment.

Fracture:

Proton pump inhibitors, especially is used in high doses and over long durations (> 1 year), may modestly increase the risk of hip, wrist and spine fracture, predominantly in the elderly or in presence of other recognized risk factors. Observational studies suggest that proton pump inhibitors may increase the overall risk of fracture by 10-40%. Some of this increase may be due to other risk factors. Patients at risk of osteoporosis should receive care according to current clinical guidelines and they should have an adequate intake of Vitamin D and calcium.

Clostridium Difficile Diarrhea:

Published observational studies suggest that PPI therapy may be associated with an increased risk of Clostridium difficile associated diarrhea, especially in hospitalized patients. This diagnosis should be considered for diarrhea that does not improve. Patients should use the lowest dose and shortest duration of PPI therapy appropriate to the condition being treated.

Vitamin B12 Deficiency:

Daily treatment with any acid-suppressing medications over a long period of time (e.g. longer than 3 years) may lead to malabsorption of cyanocobalamin (vitamin B12) caused by hypo- or achlorhydria. Rare reports of cyanocobalamin deficiency occurring with acid-suppressing therapy has been reported in the literature. This diagnosis should be considered if clinical symptoms consistent with cyanocobalamin deficiency are observed.

PREGNANCY AND LACTATION

- Adequate and well-controlled studies in humans have not been done. There is no evidence on the safety of omeprazole in human pregnancy. Animal studies have revealed no teratogenic effect, but reproduction studies have revealed reduced litter weights. Avoid in pregnancy unless there is no safer alternative.
- It is not known whether omeprazole is excreted in human milk. However, because omeprazole has been shown to cause tumorigenic and carcinogenic effects in animals, a decision should be made on whether nursing should be discontinued or the medication withdrawn, taking into account the importance of the omeprazole to the mother.

DRUG INTERACTIONS

- Since omeprazole may increase gastrointestinal pH, concurrent use with ampicillin esters, iron salts or ketoconazole may result in a reduction in absorption of these medications.
- Inhibition of the cytochrome P-450 enzyme system by omeprazole, especially in high doses, may cause a decrease in the hepatic metabolism of anticoagulants (coumarin or indandione-derivative) or diazepam or phenytoin, which may result in delayed elimination and increased blood concentrations, when these medications are used concurrently with omeprazole.
- Concurrent use of omeprazole with bone marrow depressants may increase the leukopenic and/or thrombocytopenic effects of both these medications; if concurrent use is required, close observation for toxic effects should be considered.

MAIN SIDE/ADVERSE EFFECTS

- Cases of hematological abnormalities, specifically anaemia (unusual tiredness or weakness), eosinopenia, leukocytosis (sore throat and fever), neutropenia (continuing ulcers or sore in mouth), pancytopenia or thrombocytopenia (unusual bleeding or bruising), haematuria (bloody urine), proteinuria (cloudy urine), urinary tract infection (difficult, burning, or painful urination, frequent urge to urinate or bloody or cloudy urine).
- Abdominal pain and colic.
- Asthenia (unusual tiredness, muscle pain); central nervous system (CNS) disturbances, specifically dizziness, headache, somnolence (unusual drowsiness), or unusual tiredness; chest pain, gastrointestinal disturbances, specifically acid regurgitation (heartburn), constipation, diarrhea or loose stools, flatulence (gas), or nausea and vomiting; skin rash or itching.
- Gastrointestinal disorders: Microscopic colitis - Frequency 'not known'

Subacute Cutaneous Lupus Erythematosus (SCLE)

Skin and subcutaneous tissue disorders
Frequency 'not known': Subacute Cutaneous Lupus Erythematosus

Interstitial Nephritis

Renal and urinary disorders: interstitial nephritis

Hypomagnesaemia

Metabolism and nutritional disorders
Frequency 'not known': hypomagnesaemia

Fracture

Musculoskeletal disorders
Frequency 'uncommon': fracture of the hip, wrist or spine.

Clostridium Difficile Diarrhea

Infections & infestations: Clostridium difficile associated diarrhea

Fundic Gland Polyps (Benign)

Gastrointestinal disorders
Frequency 'common': fundic gland polyps (benign)

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Vitamin B12 Deficiency

Metabolic/nutritional: Vitamin B12 deficiency

OVERDOSAGE AND TREATMENT

Clinical features: No information available on the effects of overdosage in man.

Treatment for overdosage: Since there is no specific antidote, treatment should be symptomatic and supportive.

DOSAGE AND ADMINISTRATION

Omeazole capsules are recommended to be taken immediately before meals, preferably in the morning and swallowed whole with liquid. For patients with swallowing difficulties the capsule might be opened and the contents swallowed or suspended in a slightly acidic fluid e.g juice, soured milk, or non-carbonated water. The dispersion should be taken immediately or within 30 minutes.

Alternatively patients can suck the capsule and swallow the pellets with liquid. The pellets must not be chewed or crushed.

Usual adult and adolescent dose:

- **Reflux Oesophagitis:** The recommended dosage is Omeazole 20 mg once daily. Symptom resolution is rapid and in most patients healing occurs within 4 weeks. For those patients who may not be fully healed after the initial course, healing usually occurs during a further 4 weeks treatment period. In patients with severe reflux oesophagitis, Omeazole 40 mg once daily is recommended and healing is usually achieved within 8 weeks.
- **Duodenal Ulcer/ Benign Gastric Ulcer:** Oral, 20 mg once a day. The dosage can be increased to 40 mg once a day for duodenal/gastric ulcer refractory to other treatment regimens. If healing of gastric ulcer has not occurred within 4 weeks, an additional 4 weeks of treatment is recommended. Long term therapy for patients with history of recurrent duodenal ulcer is recommended at a dosage of 20 mg Omeazole once daily, up to one year.
- **Gastric hypersecretory (e.g., Zollinger-Ellison):** Oral, 60 mg once a day, the dosage being adjusted as needed, and therapy continued for as long as clinically indicated.
- Dose adjustment is not required in the elderly.
- **Impaired renal function:** Dose adjustment is not required in patients with impaired renal function.
- **Impaired hepatic function:** As bioavailability and plasma half-life of omeprazole are increased in patients with impaired hepatic function, a daily dose of 20 mg may be sufficient.

Note: The information given here is limited. For further information, consult your doctor or pharmacist.

Storage:

Store below 25°C. Protect from moisture.

Presentation/Packing:

Capsule 20 mg x Blisters of 2 x 7's, 4 x 7's, 10 x 7's and 20 x 7's.

Product Registration Holder: HOVID Bhd.

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(Jalan Kuala Kangsar),
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Manufactured by: HOVID Bhd.

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31200 Chemor, Perak, Malaysia.

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